



Laboratory Investigation Report

Patient Name	: Suman Rani	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 28 Y 0 M 0 D /F	OP/IP No	: OP/101822
Max ID/Mobile	: 454584/8752005200	Collection Date/Time	: 14/Sep/2021 12:23PM
Lab ID	: 1266092100314	Receiving Date	: 14/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 15/Sep/2021
Passport No.	:		

Clinical Biochemistry

KFT Profile with Calcium,Uric Acid*, Serum

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
Urea Enzymatic Rate (Urease)	19.7	mg/dL	17.12 - 55.64
Creatinine Rate-Jaffe	0.55	mg/dl	0.61 - 1.24
eGFR MDRD	131.61	ml/min/1.73 m ²	
Uric Acid Enzymatic Trinder	5.60	mg/dl	2.6 - 8.0
Calcium (Total) ISE Indirect	8.78	mg/dl	8.9 - 10.3
Sodium ISE Indirect	140.1	mmol/L	136 - 144
Potassium ISE Indirect	4.51	mmol/L	3.6 - 5.1
Chloride ISE Indirect	101.6	mmol/l	101-111
Bicarbonate ISE Indirect	22.5	mmol/l	22-32

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs $\leq$ 60ml / min /1.73 m².MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min /1.73 m² - Kidney failure



SIN No:B2B899483, Test Performed at :896 - Motherland Hospital, Motherland hospital Sec-119

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CIN No.: L72200MH2001PLC322854

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Clinical Biochemistry

Liver Function Test Profile,Serum*

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
Total Protein	7.30	g/dl	6.5 - 8.1
Biuret			
Albumin	4.7	g/dl	3.5 - 5.0
BCP			
Globulin	2.6	g/dl	2.3 - 3.5
Calculated			
A.G. ratio	1.8		1.2 - 1.5
Calculated			
Bilirubin (Total)	0.48	mg/dl	0.3 - 1.2
Diazo			
Bilirubin (Direct)	0.19	mg/dl	0.1 - 0.5
Diazo			
Bilirubin (Indirect)	0.29	mg/dL	0.1 - 1.0
Calculated			
SGOT- Aspartate Transaminase (AST)	28.65	U/L	10 -35
UV without P5P			
SGPT- Alanine Transaminase (ALT)	35.33	U/L	< 40
Kinetic Rate using LDH			
Alkaline Phosphatase	70.85	U/L	32 - 91
PNP AMP Buffer			
GGTP (Gamma GT), Serum	32.9	U/L	7 - 50
Enzymatic Rate			

Kindly correlate with clinical findings

*** End Of Report ***



Page 2 of 12

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Clinical Biochemistry

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
APTT	30.3	Sec	24.9 - 33.3
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	12.6	Sec	12.1 - 15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.11		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.



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Hematology

Platelet Count*, EDTA

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
Platelet Count	254.4	10~9/L	150-410
Electrical Impedance			



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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	14/Sep/2021 12:26PM	Unit	Bio Ref Interval
Haemoglobin	13.00	g/dl	12.0 - 15.0
Modified cyanmethemoglobin			
Packed Cell, Volume	38.1	%	40-50
Calculated			
Total Leucocyte Count (TLC)	7.04	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	4.65	10~12/L	3.8-4.8
Electrical Impedance			
MCV	82.0	fL	83-101
Electrical Impedance			
MCH	28.0	pg	27-32
Calculated			
MCHC	34.1	g/dl	31.5-34.5
Calculated			
Platelet Count	254.4	10~9/L	150-410
Electrical Impedance			
MPV	11.57	fL	7.8-11.2
Calculated			
RDW	13.9	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	64	%	40-80
Lymphocytes	24	%	20-40
Monocytes	09	%	2-10
Eosinophils	03	%	1-6
Basophils	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	4.51	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.7	10~9/L	1.0-3.0
Absolute Monocyte Count	0.63	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.21	10~9/L	0.02-0.5



Page 6 of 12

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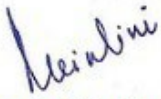
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Hematology

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Mrinalini Priyadarshini
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Page 7 of 12

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Hematology Special

Test Name	Result	Unit	Bio Ref Interval
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PDF Attached

Haemoglobin HPLC, EDTA

Lab Number	7817/2021		
Hb F	0.3	%	0.0-1.0
P2	4.5		
P3	5.9	%	<6.0
HbA0 (non-glycated)	85.1	%	80-90
Hb A2	2.8	%	2.0-3.5

RBC Indices

Haemoglobin. Non- Haemoglobincyanide	13.3	gm/dl	12-15
RBC Count. Electrical Impedance	5.04	10~12/L	4.5-5.5
MCV. Electrical Impedance	83.8	fL	83-101
MCH. Calculated	26.4	pg	27-32
RDW. Calculated	15.6	%	11.5-14.5

Interpretation (HPLC)

Normal Study

Comment

(Syn: - High Performance Liquid Chromatography, Hb HPLC)

Hb HPLC is used for screening of Beta Thalassemia and detection of many other haemoglobin variants (Hemoglobin E, D Punjab, Sickle cell anaemia etc.) along with their concentration to establish zygosity.

Normal HbA2 values can be seen in Alpha Thalassemia trait, silent/ atypical Beta Thalassemia and Beta Thalassemia with iron deficiency.

A repeat Hb HPLC should be performed after replenishing iron stores in cases with microcytic hypochromic indices.

Advise: -

If there is absence of iron deficiency & persistence of microcytic hypochromic indices, then the possibility of Alpha Thalassemia trait or silent/ atypical Beta Thalassemia trait cannot be ruled out. Hb HPLC of parents and molecular sequencing of globin genes are recommended in these cases.





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Hematology Special

Test Name	Result	Unit	Bio Ref Interval
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In individuals with beta thalassemia trait and carriers of haemoglobin variants –

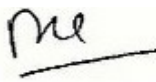
1. Familial study is recommended.
2. In cases of young/ pregnant females - Hb HPLC testing of spouse is strongly recommended.
3. Genetic counselling

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology





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Immunoassay

Estradiol (E2),Serum

Date	14/Sep/2021 12:23PM	Unit	Bio Ref Interval
Estradiol CLIA	27	pg/mL	

Ref Range

Male	20 - 75
Post - Menopausal (Female)	20 - 88
Estradiol -Total (Non - Pregnant Females)	
Mid Follicular Phase	24 - 114
Mid - Luteal Phase	80 - 273
Periovulatory	62 - 534





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Immunoassay

FSH - Follicle Stimulating Hormone, Serum

Date	14/Sep/2021 12:22PM	Unit	Bio Ref Interval
Follicle Stimulating Hormone	7.53	mIU/mL	
CLIA			

Ref. Range

Adult Male	1.27 - 19.26
Adult Female :	
Follicular	3.85 - 8.78
Midcycle Peak	4.54 - 22.51
Luteal Phase	1.79 - 5.12
Post Menopausal (>50 Yrs)	16.74 - 113.59

Interpretation

Increased in primary gonadal failure, ovarian or testicular agenesis, Klinefelter's syndrome, Reifenstein's syndrome, castration, alcoholism, menopause, orchitis, gonadotropin-secreting pituitary tumors.

Decreased in anterior hypofunction, hypothalamic disorders, pregnancy, anorexia nervosa, polycystic ovarian disease, hemochromatosis, sickle cell anaemia, severe illness, hyperprolactinemia.

Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.





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Immunoassay

LH-Luteinizing Hormone , Serum

Date	14/Sep/2021 12:22PM	Unit	Bio Ref Interval
Luteinizing Hormone CLIA	13.78	mIU/mL	

Ref Range

LH(Male-Adult)	Reference Range
	1.24-8.62
LH (Female-Adult)	
Follicular	2.12-10.89
Mid Cycle Peak	19.18-103.03
Luteal Phase	1.2-12.86
Post Menopausal (>50 Year)	10.87-58.64

Interpretation

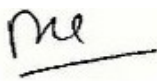
Increased in Primary gonadal dysfunction, polycystic ovarian syndrome (LH/FSH ratio is high in 60% cases), post-menopause, and pituitary adenoma.
Decreased in pituitary or hypothalamic impairment, isolated gonadotropic deficiency associated with anosmia or hyposmia (Kallmann's syndrome), anorexia nervosa, isolated LH deficiency ("fertile eunuch"), sever stress, malnutrition, and sever illness.
Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.

Kindly correlate with clinical findings

*** End Of Report ***



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Patient Data

Sample ID: **B2B899483**
Patient ID: 1414909/7817/21
Name:
Physician:
Sex:
DOB:
Comments:

Analysis Data

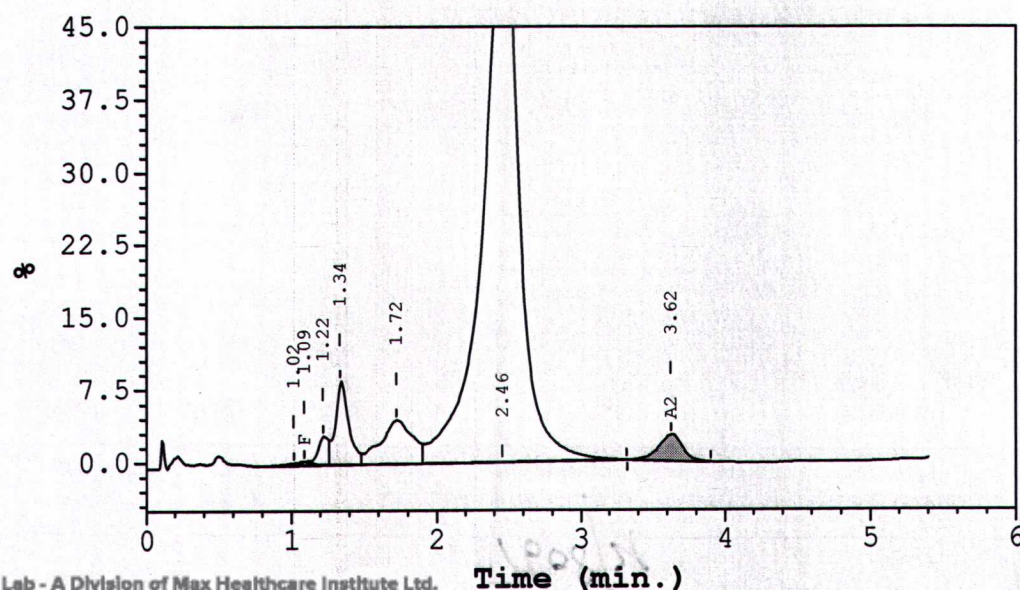
Analysis Performed: 15/09/2021 13:40:13
Injection Number: 511U
Run Number: 15
Rack ID: 0003
Tube Number: 9
Report Generated: 15/09/2021 15:12:57
Operator ID:

Peak Name	Calibrated Area %	Area %	Retention Time (min)	Peak Area
Unknown	---	0.1	1.02	1737
F	0.3	---	1.09	4380
Unknown	---	1.4	1.22	22363
P2	---	4.5	1.34	74951
P3	---	5.9	1.72	97446
Ao	---	85.1	2.46	1404358
A2	2.8	---	3.62	44254

Total Area: 1,649,489

F Concentration = 0.3 %**A2 Concentration = 2.8 %**

Analysis comments:



Max Lab - A Division of Max Healthcare Institute Ltd.
Max Super Specialty Hospital, Saket (West Block)
1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.
CIN No.: L72200MH2001PLC322854
www.maxlab.co.in

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/ laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and male fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

**Laboratory Investigation Report**

Patient Name	: Suman Rani	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 28 Y 0 M 0 D /F	OP/IP No	: OP/101822/
UHID	: 454584	Collection Date/Time	: 14/Sep/2021 12:26PM
Lab ID	: 1266092100314	Receiving Date	: 14/Sep/2021
Ref Doctor	: Dr. Richika Sahay Shukla	Reporting Date	: 15/Sep/2021
Passport No.	: {Passport}		

Microbiology**High Vaginal Swab Culture And Sensitivity**

Method : Culture/ID & Sensitivity by Vitek 2

Result: No pathogen isolated.

Kindly correlate with clinical findings

***** End Of Report *******Dr. Bansidhar Tarai, M.D.**Associate Director
Microbiology & Molecular Diagnostics**Dr. Poornima Sen, M.D.**

Consultant - Microbiology

**Dr. Madhuri Somani, M.D., DNB**

Consultant - Microbiology

SIN N **MAX Lab - A Division of Max Healthcare Institute Ltd.**

Book Max Super Specialty Hospital, Saket (West Block) Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 875201, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

The test report can be verified by scanning the Q R Code on top of the page

Test Report **www.maxlab.co.in** Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

